

Proposal for State-Wide Swachhta Assessment of Hospitals & Medical Colleges in Gujarat

COMPREHENSIVE SWACHHTA AUDIT BASED ON CLEANLINESS, HYGIENE, AND SANITATION FRAMEWORK

PREPARED BY: QUALITY COUNCIL OF INDIA

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Introduction

Building on the Quality Council of India's (QCI) extensive experience in healthcare facility assessments, this proposal outlines a comprehensive three-month programme to audit and certify 64 strategically selected hospitals across districts of Gujarat using the **Swachhta Audit Checklist framework**.

This assessment is specifically aligned with the **Swachhta Audit Checklist for Cleanliness, Hygiene, and Sanitation** to ensure fundamental cleanliness standards are achieved across Gujarat's healthcare system.

About Quality Council of India

The Quality Council of India (QCI) is an autonomous body set up by the Ministry of Commerce and Industry, Government of India, jointly with the Indian Industry represented by the three premier industry associations, i.e., Associated Chambers of Commerce and Industry of India (ASSOCHAM), Confederation of Indian Industry (CII), and Federation of Indian Chambers of Commerce and Industry (FICCI). QCI was instituted to establish and operate a national accreditation structure and promote quality through National Quality Campaign. QCI is pivotal in conducting a third-party assessment of products, services, and processes. Thereby helping to propagate adoption and adherence to quality standards in all critical activities, including education, healthcare, environment protection, governance, social sectors, infrastructure sector, etc. The Department of Promotion of Industry and Internal Trade (DPIIT), Ministry of Commerce and Industry, is the nodal point for all matters related to QCI. The Hon'ble Prime Minister of India appoints the Chairman of QCI.

QCI Relevant Past Experience (2019)

- **Scope:** 653 private hospitals assessed nationwide using structured questionnaire
- **Methodology:** 0-2 scoring rubric with trained assessors
- **Impact:** National promotion of Swachhta culture and heightened community awareness

Strategic Objectives

- Institutionalize basic hygiene and sanitation standards across Gujarat's healthcare sector
- Reduce Healthcare Associated Infections (HAI) by at least 20% from baseline within 12 months
- Position Gujarat as a national benchmark for hospital cleanliness using Swachhta standards
- Create sustainable cleanliness practices in public health facilities

Scope of Work: Swachhta Assessment Framework

The assessment will be conducted based on the **Swachhta Audit Checklist: Cleanliness, Hygiene, and Sanitation** covering the following core areas: (To be Checked)

1. General Cleanliness and Upkeep

i. Open Areas & Infrastructure

1. Solid, organic, or any other types of waste.
2. Waterlogging due to faulty drainage or leakages.
3. Spitting on walls, chipping plaster/paints.
4. Garbage/waste piling.

ii. Water Logging Prevention (Area-Specific)

1. **Patient Care Areas:** Water accumulation in wards, OPD, emergency, ICU, OTs.
2. **Hostel Areas:** Water logging in corridors, rooms, common areas, mess halls, study rooms.
3. **Kitchen/Food Areas:** Water logging in kitchens, mess, food storage.
4. **Outdoor Campus:** Stagnation in courtyards, pathways, gardens, sports facilities.
5. **Basement/Underground:** Accumulation in basements, parking, storage.
6. **Roof/Terrace:** Proper drainage on rooftops, terraces, balconies.

iii. Pest & Animal Control

1. Stray animals (dogs, cats, cattle, rodents, insects).
2. Implementation of pest control measures.

3. Junk Material Removal

- a. Unused/condemned articles/records in patient areas (OPD, wards).
- b. Junk in open areas, corridors, under stairs, rooftops, balconies.
- c. Junk in critical areas (Labour room, OT, Injection room, Dressing room).

4. Safety and Accident Prevention (Slipping Hazards)

a. Floors: Kept dry and free from spills/grease/debris in all areas (wards, corridors, labs, kitchens, toilets).

b. Wet Areas: "Wet Floor" signage used in prone zones (after cleaning, near water sources).

i. Common Areas (Lounges)

1. Floors, walls, furniture clean (no dirt, grease, stains, cobwebs).
2. Regular cleaning schedule implemented.
3. Proper waste disposal.

ii. Kitchen Areas

1. Cleanliness of kitchen areas, dining halls, serving, and food prep areas.
2. Clean crockery, cutlery, serving utensils.
3. Proper kitchen waste disposal (wet/dry segregation).

iii. Toilets & Bathrooms

1. Clean toilets, washbasins, fittings, floors (no dirt, stains, water accumulation).
2. Foul smell.
3. Sinks/cisterns cleaned every 2 hours (or as needed).
4. Floors dry and residue-free.
5. Availability of soap/handwash and toilet paper.

4. Water, Sanitation, and Hygiene (WASH)

- a. Maintained water supply (no leaking taps, overflowing tanks, dysfunctional cisterns).
- b. Water tanks cleaned quarterly (minimum).

5. Drainage and Sewage Management

- a. Closed drainage system (or properly covered open drains).
- b. Adequate drain gradient to prevent accumulation.

- c. Blocked/overflowing drains.
- d. Drains cleaned weekly (minimum).

6. Area-Specific Drainage Systems

- a. **Hospital Buildings:** Functional drainage in OPD, emergency, ICU, OTs, Labour rooms.
- b. **Hostels:** Proper drainage in blocks, common areas, mess halls.
- c. **Laboratories:** Adequate drainage with proper lab wastewater disposal.
- d. **Kitchen/Food Areas:** Grease traps and drainage in kitchens/mess.
- e. **Toilets/Bathrooms:** Functional drainage (no overflow/blockage).
- f. **Laundry Areas:** Proper drainage for washing/disposal.
- g. **Outdoor Areas:** Stormwater drainage for roads, pathways, parking.

7. Hand Hygiene

- a. Sinks with running water at point of use (nursing stations, OPD, OT, classrooms, hostels).
- b. Handwashing instructions displayed.
- c. Staff/students follow 6-step handwashing.
- d. Alcohol-based hand rub available.

8. Kitchen Services (Hospital & Hostel Mess)

- a. Smoke-free and fly-proofed kitchen.
- b. Staff maintains meticulous personal hygiene.

9. Hygiene Promotion

10. Staff Hygiene and Dress Code

- a. Regular monitoring of hygiene practices for food handlers and housekeeping staff.

10. Continuous staff performance monitoring system

- a. Adherence to Protocols, absenteeism and punctuality, performance

Assessment Methodology

The assessment will cover 64 hospitals across Gujarat, including medical colleges, district hospitals, and sub-district hospitals. First, a detailed checklist (questionnaire) is made to cover all cleanliness, hygiene, and sanitation points as per Swachhta standards. Assessors are given proper training on how to use it.

Once ready, trained teams visit each of the 64 hospitals. They follow a set schedule for the day, including meetings, inspection of different hospital areas, checking documents, and talking to staff. During their visits, they score each area based on what they observe, making sure to note strengths and gaps.

All this information is entered into a digital dashboard so progress can be tracked, and any problems are spotted quickly. To keep the data reliable, regular quality checks and reviews take place. After all visits are complete, results are collected and analysed for each hospital. Detailed reports for all 64 hospitals are then prepared, showing where each hospital is doing well and where improvements are needed.

This step-by-step process helps make sure that every hospital is fairly assessed, and gives clear suggestions for making them cleaner and safer for everyone.

Implementation Timeline

Month	Activities	Deliverables
Month 1	Preparation, training, tool customization	Trained assessors, mobile app, hospital briefings
Month 2	Field assessments (60% completion)	60% hospital assessments completed
Month 3	Field assessments (100%), gap analysis	All assessments, gap reports, Corrective action plans, progress monitoring

Phase 1: Preparatory Activities (15 Days to 30 Days)

1. **Hospital Selection:** Confirmation of hospitals across 33 districts
2. **Assessor Training:** Training based on Swachhta Audit Checklist framework
3. **Tool Development:** Customization of mobile assessment application
4. **Stakeholder Engagement:** Briefing sessions with hospital administrators

Phase 2: Swachhta Assessment Process (45 Days)

Assessment Team Composition

- **Large Hospitals** (>150 beds): 4 man-days or completion of the assessment, whichever occurs first.
- **Medium Hospitals** (50-150 beds): 3 man-days or completion of the assessment, whichever occurs first.
- **Small Hospitals** (<50 beds): 2 man-days or completion of the assessment, whichever occurs first.

• Daily Assessment Schedule

Time	Activity	Duration
09:00-10:00	Opening meeting & facility overview	1 hour
10:00-12:00	General cleanliness assessment	2 hours
12:00-14:00	Area-specific evaluations	2 hours
14:00-15:00	Basic systems review	1 hour
15:00-16:00	Staff interviews & documentation	1 hour
16:00-17:00	Feedback	1 hour

Areas to be Assessed

- **Patient Care Areas:** Wards, nursing stations, patient rooms
- **Terrace Areas:** Water storage tank areas
- **Circulation Areas:** Corridors, waiting areas, stairs
- **Support Areas:** Kitchen, laundry, storage areas
- **Sanitation Areas:** Toilets, bathrooms, drainage systems
- **Outdoor Areas:** Hospital compound, parking, gardens
- **Open Areas**

Phase 3: Gap Analysis and Feedback (10 Days)

Immediate Feedback (Within 48 Hours)

- **Color-Coded Scoring:**
 - **Green:** Areas meeting Swachhta standards
 - **Yellow:** Areas requiring improvement
 - **Red:** Areas needing urgent intervention

Gap Report Components

- **Priority Issues:** Critical cleanliness and hygiene gaps
- **Quick Fixes:** Immediate actions that can be taken within 7 days
- **Medium-term Actions:** Improvements requiring 30-60 days
- **Long-term Strategies:** Systematic improvements over 3-6 months

Phase 4: Improvement Support (5 Days)

- **Best Practice Sharing:** Documentation of exemplary practices

Phase 5: Verification and Recognition (15 Days)

- **Report Compilation:** State-level comprehensive report

Swachhta Scoring Framework

Assessment Criteria and Weights

Thematic Area	Weight	Key Components
General Cleanliness	40%	Open areas, infrastructure, waste absence
Water Logging Prevention	20%	Drainage systems, area-specific water management
Floor & Surface Maintenance	20%	Floor cleanliness, surface conditions, signage
Basic Hygiene Standards	15%	Hand washing, toilet facilities, staff hygiene
Basic Sanitation Systems	5%	Water quality, waste disposal basics

Scoring Scale

- **2 Points:** Full compliance with Swachhta standards
- **1 Point:** Partial compliance, minor gaps identified
- **0 Points:** Non-compliance, major intervention required

Levels

- **Swachhta Excellence (85-100%):** Outstanding cleanliness standards
- **Swachhta Compliant (70-84%):** Good cleanliness with minor gaps
- **Swachhta Improving (50-69%):** Basic standards met, significant improvement needed
- **Swachhta Critical (Below 50%):** Urgent intervention required

Expected Impact Analysis

Quantitative Benefits

1. Cleanliness Standards:

- **Baseline:** Current variable cleanliness levels
- **Target:** 85% hospitals achieving Swachhta Compliant status
- **Impact:** Consistent cleanliness across Gujarat healthcare system

2. Cost Optimization:

- **Maintenance Costs:** 20-25% reduction through preventive measures
- **Cleaning Efficiency:** 30% improvement in cleaning protocols
- **Resource Utilization:** Better allocation of cleaning resources

Qualitative Benefits

1. **Public Trust:** Enhanced confidence in public healthcare system
2. **Staff Pride:** Improved workplace environment and professional satisfaction
3. **Community Health:** Better hygiene awareness in catchment areas
4. **System Strengthening:** Robust cleanliness monitoring systems

Risk Mitigation Strategies

Risk	Level	Mitigation Strategy
Hospital Non-cooperation	Medium	Early stakeholder engagement, government backing
Assessment Quality Variation	Low	Standardized training
Resource Constraints	Low	Phased approach, prioritized interventions
Technical Challenges	Low	Mobile app testing, offline capabilities

Deliverables

Primary Deliverables

1. **Individual Assessment Reports** with color-coded gap analysis
2. **Comprehensive State Report** with recommendations and best practices

Secondary Deliverables

1. **Best Practice Collection** showcasing exemplary hospitals

Budget Considerations

Element	Sub Element	Man-Days	Total Cost
Assessment Cost	64 Hospitals Visit	64	27,00,000
In House Resource Cost	Project Execution Team (Analysis, Scoring, Quality Check, Framework Development)	-	11,40,000
IT	Data Collection Mobile Application	-	50,000
Admin	12%	-	4,66,800
Total	-	-	43,56,800*

***Taxes extra as applicable (above cost is excluding GST)**

Payment Breakup

Sr. No.	Target or Milestones achieved	Percentage of contract value
1	Award of Work Order	30%
2	Completion of Field Assessment	50%
3	Submission of Final Report	20%

Conclusion

This proposal represents a focused approach to establishing fundamental cleanliness standards across Gujarat's healthcare system using the proven Swachhta Audit Checklist framework. By concentrating on basic yet critical aspects of hospital cleanliness, hygiene, and sanitation, we can achieve sustainable improvements that directly impact patient safety, staff satisfaction, and public trust.

The simplified yet comprehensive approach ensures that all 64 hospitals can achieve measurable improvements in cleanliness standards, creating a foundation for broader quality improvement initiatives in Gujarat's healthcare system.

Success of this Swachhta assessment initiative will establish Gujarat as a model state for healthcare cleanliness and provide a replicable framework for other states across India.

Annexure 1

List of Hospitals to be Assessed							
Sr. No.	Region	District Name	DH or SDH	Name of Hospital	No. of Sanctioned Beds as per Govt. Notification	No. of Physical (Functional) Beds	Construction Status
1	Ahemdabad	Ahmedabad	SDH	General Hospital, Rukshmaniben	100	60	
2	Ahemdabad	Ahmedabad	SDH	Sub District Hospital, Viramgam	75	50	
3	Ahemdabad	Anand	SDH	S.S.Hospital, Petlad	144	144	
4	Ahemdabad	Kheda	SDH	Sub District Hospital, Dakor	100	100	
5	Ahemdabad	Surendrangar	DH	M.G. General Hospital, Surendranagar	150	150	
6	Ahemdabad	Surendrangar	SDH	Raj Rayjitsinh Govt. Hospital, Dhrangadhra	104	56	
7	Ahemdabad	Surendrangar	SDH	R.R. Hospital, Limbdi	212	120	
8	Ahemdabad	Surendrangar	SDH	General Hospital, Patdi	100	50	Construction under planning
9	Gandhinagar	Mehsana	SDH	Sub District Hospital, KADI	75	50	
10	Gandhinagar	Mehsana	SDH	Cottage Hospital, Unjha	100	60	
11	Gandhinagar	Mehsana	SDH	General Hospital, Visnagar	145	89	
12	Gandhinagar	Mehsana	SDH	General Hospital, Vijapur	100	40	
13	Gandhinagar	Mehsana	SDH	General Hospital, Satlasana	100	30	
14	Gandhinagar	Patan	SDH	Sub District Hospital, Siddhpur	150	120	
15	Gandhinagar	Patan	SDH	Sub District Hospital, Radhanpur	100	76	
16	Gandhinagar	BK	SDH	Sub District Hospital, Tharad	100	50	
17	Gandhinagar	BK	SDH	Cottage Hospital Ambaji GIA	100	50	
18	Gandhinagar	BK	SDH	Sub District Hospital, Bhabhar	75	30	
19	Gandhinagar	SK	SDH	Sub District Hospital, Khedbrahma	150	135	
20	Gandhinagar	SK	SDH	Sub District Hospital, Idar	100	100	
21	Gandhinagar	Aravalli	SDH	Sub District Hospital, Bhiloda	125	75	
22	Vadodara	Vadodara	DH	General Hospital Jamnabai, Vadodara	213	168	
23	Vadodara	Vadodara	SDH	Sub District Hospital, Savli	100	76	
24	Vadodara	Vadodara	SDH	Sub District Hospital, Karjan	100	30	Construction under planning
25	Vadodara	Chhota Udepur	SDH	Sub District Hospital, Kwant	100	30	Construction under planning
26	Vadodara	Chhota Udepur	SDH	Sub District Hospital, Bodeli	100	30	
27	Vadodara	Mahisagar	SDH	Sub District Hospital, Santrampur	150	56	
28	Vadodara	Dahod	SDH	Shree J.S.Chauhan Govt.Hospital, Devgadhi Baria	150	90	
29	Vadodara	Dahod	SDH	Sub District Hospital, Jhalod	75	50	Construction under planning
30	Vadodara	Narmada	SDH	Sub District Hospital, Dediapada	50	50	
31	Vadodara	Narmada	SDH	Sub District Hospital, Gardudheshwar	50	30	
32	Vadodara	Narmada	SDH	Sub District Hospital, Kevadia	50	50	
33	Vadodara	Bharuch	SDH	Sub District Hospital, Jambusar	75	50	
34	Surat	Surat	SDH	Old Civil Hospital, Surat	100	50	
35	Surat	Surat	SDH	Sub District Hospital, Mandvi	150	56	
36	Surat	Surat	SDH	Sub District Hospital, Bardoli	75	50	
37	Surat	Surat	SDH	Sub District Hospital, Umarpada	100	100	Construction under planning
38	Surat	Vyara	DH	General Hospital, Vyara	150	235	
39	Surat	Vyara	SDH	Sub District Hospital, Songadh	100	50	
40	Surat	Vyara	SDH	Sub District Hospital, Nizar	100	30	
41	Surat	Navsari	SDH	Cottage Hospital, Vansda	111	111	
42	Surat	Navsari	SDH	Mengushi Hospital, Bilimora	30	30	
43	Surat	Valsad	SDH	State Hospital, Dharampur	150	200	
44	Surat	Dang	DH	General Hospital, Ahwa	200	200	
45	Bhavnagar	Bhavnagar	SDH	M. G. Hospital, Mahuva	200	85	Construction under planning
46	Bhavnagar	Botad	DH	General Hospital, Botad	100	60	
47	Bhavnagar	Amreli	SDH	Government Hospital Savarkundla	200	80	
48	Bhavnagar	Amreli	SDH	Government Hospital Rajula	100	100	
49	Bhavnagar	Junagadh	SDH	Sub District Hospital, Kesod	75	75	
50	Bhavnagar	Junagadh	SDH	Sub District Hospital, Visavadar	100	16	Construction under planning
51	Bhavnagar	Girsomnath	SDH	Sub District Hospital, Una	100	46	Construction under planning
52	Bhavnagar	Girsomnath	SDH	Sub District Hospital, Gir Gaghada	100	30	Construction under planning
53	Rajkot	Rajkot	DH	P. K. General Hospital, Rajkot	300	115	
54	Rajkot	Rajkot	SDH	Sub District Hospital, Gondal	150	115	
55	Rajkot	Rajkot	SDH	Sub District Hospital, Dhoraji	81	56	
56	Rajkot	Rajkot	SDH	Sub District Hospital, Jetpur	93	68	Construction under planning
57	Rajkot	Jamnagar	SDH	Sub District Hospital, Jamjodhpur	100	20	Construction under planning
58	Rajkot	Morbi	SDH	Sub District Hospital, Wankaner	100	70	
59	Rajkot	Morbi	SDH	Sub District Hospital, Halvad	50	50	
60	Rajkot	Devbhumi Dwarka	SDH	General Hospital, Dwarka	50	50	
61	Rajkot	Devbhumi Dwarka	SDH	General Hospital, Kalyanpur	100	30	
62	Rajkot	Kutch	SDH	Ref. Hospital Mandvi	99	69	
63	Rajkot	Kutch	SDH	Sub District Hospital, Anjar	100	40	
64	Rajkot	Kutch	SDH	Sub District Hospital, Nakhatrana	100	30	Construction under planning